

Patient:		Referring Physician:	UID: 1396A81D-089B-4E98-A099-E15AFABE4191 TCGA-FY-A3W9-01A-PR Redacted	
DOB	Age	Gender		
Ref#:	Hosp#:	Provider Group:		
Date of Service:	Date Received:	Outpatient	Case #	
			Date Reported:	

FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

THYROID GLAND, RIGHT LOBE, LOBECTOMY:

- MULTIPLE ADENOMATOID NODULES (MULTINODULAR GOITER).
- NO ADDITIONAL FOCI OF PAPILLARY CARCINOMA IDENTIFIED.
- NO ADDITIONAL LYMPH NODES IDENTIFIED.
- SEE PATHOLOGIC TUMOR STAGING SUMMARY BELOW.

PATHOLOGIC TUMOR STAGING SUMMARY (INCLUDES INFORMATION FROM PRIOR LEFT LOBECTOMY SPECIMEN,):

Histologic type: Papillary thyroid carcinoma, follicular variant. 99% follicular variant, per TSS, 5/11/12 lw
 Primary tumor: pT2 (2.3 cm in greatest dimension, limited to the thyroid).
 Regional lymph nodes: pN0 (one lymph node, negative for metastasis).
 Distant metastasis: Not applicable.
 Pathologic stage: II.
 Lymphovascular invasion: Not identified.
 Margin status: R1 (tumor focally extends to the inked margin).

Tumor Staging Information

Data derived from current specimen. Staging in accordance with or modified from AJCC Cancer Staging Handbook, Ed. and CAP protocol. Also includes information from prior left lobectomy specimen.

ICD-O-3
 carcinoma, papillary, follicular variant
 8340/3

site: thyroid, NOS
 C73.9
 5-14-12 ro

Criteria	Yes	No
Diagnostic Discrepancy		
Primary Tumor Site Discrepancy		
IPAA Discrepancy		
Prior Malignancy History		
Qual/Synchronous Primary Noted		
Case Included	QUALIFIED	DISQUALIFIED
Reviewed (Initials)		
Date Reviewed	5/10/12	

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Procedure:
Specimen type:
Specimen integrity:
Specimen size:

Total thyroidectomy.
Thyroid gland, left and right lobes.
Left and right lobes submitted separately.
Left lobe: 4.8 x 2.3 x 1.8 cm
Right lobe: 4.5 x 1.8 x 1.3 cm

TUMOR FEATURES:

Tumor size:
Tumor focality:
Tumor laterality:
Histologic type:
Lymphovascular invasion:
Tumor capsule:
Tumor capsular invasion:
Extrathyroidal extension:

2.3 cm in maximum dimension
Unifocal.
Left.
Papillary thyroid carcinoma, follicular variant
Not identified.
Partially encapsulated.
Present, minimal.
Not identified.

LYMPH NODES:

One lymph node, negative for metastasis

MARGIN EVALUATION:

Distance to closest margin:
Other margins:

POSITIVE (R1)
Tumor focally extends to the inked margin
Uninvolved by carcinoma

PATHOLOGIC TUMOR STAGING DESCRIPTORS:

Primary tumor:

pT2 (2.3 cm in greatest dimension, limited to the thyroid)

Regional lymph nodes:

pN0 (0/1 lymph node).

Distant metastasis:

Not applicable

Margin status:

R1 (tumor focally extends to the inked margin).

Pathologic stage:

II

Additional findings:

Multiple adenomatoid nodules (multinodular goiter).
One benign parathyroid gland.

Signed

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FINAL SURGICAL PATHOLOGY REPORT

Source of Specimen:

Right thyroid lobe

Clinical History/Operative Dx:

Thyroid cancer

Gross Description:

Received in formalin is three portions/pieces of soft tissue, including a 5 gram lobe of thyroid 4.5 x 1.8 x 1.3 cm; a strandular fragment of fatty soft tissue is 3.6 x 0.7 x 0.4 cm, and a lobular fragment of light pink and tan tissue, 0.8 x 0.4 x 0.4 cm. The smaller lobular fragment is bisected revealing glistening yellow-tan fatty appearing cut surface (A2). The strandular, fatty fragment is bisected lengthwise revealing soft to indurated yellow-gray cut surface with hemorrhagic congestion. The lobe of thyroid has one end marked by a double long suture, unassigned, and likely corresponding to the superior pole. The mid medial periphery of the lobe demonstrates an attached multilobular, purple-tan fragment, 1.8 x 0.8 x 0.6 cm, that is bisected to reveal semi-translucent yellow-tan nodules up to 0.5 cm and a 0.3 cm pink-gray nodule. This attachment is submitted bisected (A2). The posterior and anterior margin of the thyroid are now differentially inked black and blue, respectively. The specimen is serially sectioned perpendicularly through the superior-inferior long axis to reveal a meaty deep red-tan cut surface with glistening light yellow gelatinous flecks of change, and an irregular peripheral area of light tan nodularity along the mid anterior margin (A5-A6) measuring upwards of 0.8 x 0.7 x 0.3 cm. No additional lesions appreciated. The specimen is entirely submitted for microscopic evaluation in a sequential fashion: superior to inferior.

Cassette summary: A1) separate smaller fatty lobule, long fatty strand, sectioned and entirely submitted. A2) nodular attachment of possible thyroid tissue, bisected. A3-A8) right lobe of thyroid submitted superior to inferior.

Microscopic Description:

Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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Pathology Report -

Referring Physician:

DOE [REDACTED] Age: [REDACTED] Gender: [REDACTED]

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Outpatient

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FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

THYROID GLAND, LEFT LOBE, LOBECTOMY:

- PAPILLARY THYROID CARCINOMA, FOLLICULAR VARIANT.
 - 2.3 cm in greatest dimension.
 - Tumor focally extends to the inked surgical margin (slide A2).
- BENIGN ADENOMATOID NODULES (MULTINODULAR GOITER).
- ONE LYMPH NODE, NEGATIVE FOR METASTASIS.
- ONE BENIGN PARATHYROID GLAND.
- SEE COMMENT.

COMMENT: The patient's previous thyroid fine needle aspiration cytology specimen diagnosis of "suspicious for follicular neoplasm" is noted, and correlates with the current biopsy.

A complete tumor staging synopsis will be issued after review of the the completion thyroidectomy specimen.

Dr. [REDACTED] notified of the diagnosis of papillary carcinoma at approximately on [REDACTED]

Signed

Source of Specimen:

Thyroid; Left thyroid lobe

Clinical History/Operative Dx:

Neoplasm of unspecified nature of endocrine glands and other parts of nervous system

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FINAL SURGICAL PATHOLOGY REPORT

Gross Description:

Single specimen designated as left thyroid lobe. Initially received in the fresh state for Oncogenotyping studies is an 8 gram lobe of thyroid, 4.8 x 2.3 x 1.8 cm. The surfaces of the thyroid are glistening and lobular, deep red. The margins are marked blue. The specimen is serially sectioned perpendicularly through the long axis to reveal a well-circumscribed, glistening, tan nodular lesion, 2.3 x 1.4 x 1.3 cm abutting the anterior and posterior thyroid margins. Cut sections of the lesion demonstrates multiple foci of calcifications. Representative portions are submitted for Oncogenotyping studies along with "normal" thyroid parenchyma. The remaining cut sections of the thyroid demonstrate glistening, semi-translucent darkened tan-red nodules ranging from 0.5-1.2 cm. The uninvolved thyroid parenchyma is meaty deep red. The entire lobe of thyroid is submitted for microscopic evaluation with the large mass submitted in A1-A5 and the remaining thyroid tissue including nodularity in A6-A9.

Microscopic Description:

Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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AMENDED SURGICAL PATHOLOGY REPORT

Amendment Information:

This amendment is issued to add that the thyroid carcinoma does not demonstrate extrathyroidal extension, and to correct a typographical error in the comment. The diagnosis is otherwise unchanged.

Diagnosis:

THYROID GLAND, LEFT LOBE, LOBECTOMY:

- PAPILLARY THYROID CARCINOMA, FOLLICULAR VARIANT.
 - 2.3 cm in greatest dimension.
 - Tumor focally extends to the inked surgical margin (slide A3).
 - Extrathyroidal extension is not identified.
- BENIGN ADENOMATOID NODULES (MULTINODULAR GOITER).
- ONE LYMPH NODE, NEGATIVE FOR METASTASIS.
- ONE BENIGN PARATHYROID GLAND.
- SEE COMMENT.

COMMENT: The patient's previous thyroid fine needle aspiration cytology specimen diagnosis of "suspicious for follicular neoplasm" () is noted, and correlates with the current biopsy.

Please see the completion thyroidectomy specimen () for the complete tumor staging synopsis.

Dr. notified of the diagnosis of papillary carcinoma at approximately

Amendment signed

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AMENDED SURGICAL PATHOLOGY REPORT

Source of Specimen:

Thyroid; Left thyroid lobe

Clinical History/Operative Dx:

Neoplasm of unspecified nature of endocrine glands and other parts of nervous system.

Gross Description:

Single specimen designated as left thyroid lobe. Initially received in the fresh state for Oncogenotyping studies is an 8 gram lobe of thyroid, 4.8 x 2.3 x 1.8 cm. The surfaces of the thyroid are glistening and lobular, deep red. The margins are marked blue. The specimen is serially sectioned perpendicularly through the long axis to reveal a well-circumscribed, glistening, tan nodular lesion, 2.3 x 1.4 x 1.3 cm abutting the anterior and posterior thyroid margins. Cut sections of the lesion demonstrates multiple foci of calcifications. Representative portions are submitted for Oncogenotyping studies along with "normal" thyroid parenchyma. The remaining cut sections of the thyroid demonstrate glistening, semi-translucent, darkened tan-red nodules ranging from 0.5-1.2 cm. The uninvolved thyroid parenchyma is meaty deep red. The entire lobe of thyroid is submitted for microscopic evaluation with the large mass submitted in A1-A5 and the remaining thyroid tissue including nodularity in A6-A9.

Microscopic Description:

Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

Final report originally signed by

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